

1. Every patient wants to be cured.
2. There is a universal and right way to cure them.

Not requiring patient consent in treatment plans makes sense if you believe in those two points.

But that's not how medicine works.

In the last 50 years medical schools subtly shifted teaching away from treating disease and toward treating patients. That meant laying out the options of treatment plans, and then letting the patient decide the best path forward.

This trend was partly driven by patient-protection laws, partly by Katz's influential book, which argued that patients have wildly different views about what's worth it in medicine, so their beliefs have to be taken into consideration. Katz wrote:

It is dangerous nonsense to assert that in the practice of their art and science physicians can rely on their benevolent intentions, their abilities to judge what is the right thing to do ... It is not that easy. Medicine is a complex profession and the interactions between physicians and patients are also complex.

That last line is important. "Medicine is a complex profession and the interactions between physicians and patients are also complex."

You know what profession is the same? Financial advice.

I can't tell you what to do with your money, because I don't know you.

I don't know what you want. I don't know when you want it. I don't know why you want it.

So I'm not going to tell you what to do with your money. I don't want to treat you like a dentist treated Clarence Hughes.

But doctors and dentists aren't useless, obviously. They have knowledge. They know the odds. They know what tends to work, even if patients come to different conclusions about what kind of treatment is right for them.

Financial advisors are the same. There are universal truths in money, even if people come to different conclusions about how they want to apply those truths to their own finances.